

Rational Coping Statements

Many people with mental health issues, particularly anxiety sufferers or those who suffer from depression, have trouble with self-defeating thoughts and negative self-talk. This can be one of the many difficult aspects of overcoming mental illness. One of the tools that these patients are taught to use is thought stopping which is just like it sounds, but it can be difficult for the negative thoughts to be stopped when there is not something to replace them with.

That is why preparing coping statements and creating coping cards can be so very useful. This sounds like it might be too simple of a concept to really make a difference, but mental health professionals have been using this method for years and have found it to be very effective.

What are coping statements?

Coping statements are truthful positive statements used to replace the negative and untrue thoughts that take-over when you feel anxious, stressed, angry, and/or when facing other overwhelming situations.

"Remember that it won't hurt forever" is a coping statement. Being a truthful statement counter-acts negative unhelpful thinking such as " It always hurts me." Or " there is no hope, I will never get over this."

What Are Positive Coping Statements?

Coping statements are positive, truthful statements that can be used to replace untrue or negative thoughts that can take over your brain and cause you to feel more anxious, angry, and stressed than you already do. These positive statements can help you to calm down and stay in control in an overwhelming

situation. The coping statements can offer you reassurance that you can make it through any difficult situation.

Create Coping Cards

Creating coping cards is the next step. Having these statements ready to go at a moment's notice can be a big help to you. Remembering to use your coping statements can be difficult. And remembering what the coping statements are can be difficult so writing the statements down can be a big help to you. Writing down your coping statements to carry with you can help you to remember to use them.

You have a few choices when it comes to making coping cards. You can make one card with all of your positive statements so that one card will be applicable in all situations. You could make several cards with different statements for different situations. With your card or cards on your person at all times you can pull them out and read your statements to yourself whenever you need them.

Four Tips For Creating An Effective Coping Strategy

Dr. Barbara Markway, Ph.D., is a social anxiety specialist. Dr. Markway outlined **some tips for writing and using** effective coping strategies in Psychology Today.

1. Use your own words to write your statements. Do not use language that you would not use otherwise. If the statements do not feel natural, you will not be able to feel good about using them. Avoid using formal or academic language if you would not use it in your ordinary speech.
2. Write coping statements that specifically target the feelings that you have. If you have anxiety about a specific situations, speak directly to that anxiety in that situation. Speak directly to your fears and phobias. Speak directly to your anger and your resentment.

3. Be reassuring in your statements but also be truthful and realistic. Do not lie to yourself in your coping statements. If you feel pain, do not tell yourself that you do not. Instead tell yourself that it will not last forever or that you are going to feel better eventually. The more accurate a statement is, the more useful it will be to you.
4. Be brief in your statements. Short statements are easier to remember. Eventually, you may be able to remember your statements without the extra push of the card. Eventually, your card may turn into something different.

Your inner voice can sometimes lie to you, particularly when you have different kinds of mental illnesses. Those lies can make you feel more depressed or anxious or panicked than you already do. But when you recognize that these thoughts could be damaging to you, you can start to use your coping statements and bring the truth back to your mind.

Using coping statements can bring you some peace and positivity. Once you start to employ your coping statements, you may find that you start to naturally think more positive thoughts. You may also find that when you start to think positive thoughts, you also start to feel a little better in general.

How do coping statements work?

The purpose of coping statements is to put a stop to the thoughts that lead to anxiety and to replace those thoughts with realistic, rational thoughts. When these rational self-statements are practiced and learned, your brain takes over and they automatically occur. Thomas A. Richards, Ph.D., Psychologist

What are helpful coping statements?

- Stop, and breathe, I can do this
- This will pass

- I can be anxious/angry/sad and still deal with this
- I have done this before, and I can do it again
- This feels bad, it is a normal body reaction. It will pass
- This feels bad, and feelings are very often wrong
- These are just feelings, they will go away
- This won't last forever
- Short term pain for long term gain
- I can feel bad and still choose to take a new and healthy direction

What are unhelpful coping statements and actions?

What are unhealthy ways of coping with stress?

- Watching endless hours of TV.
- Withdrawing from friends or partners or, conversely jumping into a frenzied social life to avoid facing problems.
- Overeating or weight gain.
- Undereating or weight loss.
- Sleeping too much.
- Drinking too much alcohol/ Addiction
- Lashing out at others in emotionally or physically violent outbursts.

Rational coping is the systematic process of examining beliefs and testing assumptions. A question and answer dialogue fosters a critical evaluation of one's belief system. In much the same way that a scientist reviews data in testing a hypothesis, we can learn to test our own assumptions by scrutinizing the evidence for ourselves. If the data does not fit, we can reformulate our ideas and test them again. Therefore, like the scientist, we use a similar problem-solving approach, although not always in a helpful way.

Given the fact that none of us is perfect, sometimes we stubbornly act-out ways of thinking and being that are not in our

best interest. This happens primarily because our ideas about self, others, and the world often become well-rehearsed, deeply rooted and second nature. The unfortunate result is that we achieve our goals less often, and get more upset in the process. Expressed another way, it is possible for any one of us to convincingly believe and act in ways that do not foster good coping. Consider the following example:

"I must do perfectly well at my job for the boss to accept me".

Can we ever perform perfectly well at anything? After all, Olympic athletes who train most of their lives to master their sport rarely get a perfect score. If perfection is such an uncommon prize for athletes, who, by the way, make a habit of practicing, why should the rest of us be so expectant of it?

Secondly, is it necessary to have the boss's approval? Granted, it might be helpful to get it, but you cannot prove that it is necessary. Why? Because we can always unconditionally accept ourselves no matter what others say or do.

Opinions are not necessarily facts. We often presume that our own thoughts (or what others think of us) accurately and absolutely reflect reality! Part of achieving more happiness or contentment means realizing that people and circumstances do not have to live up to our expectations of them. Learning how to be more flexible in our thinking helps us to cope better.

That is where practicing rational coping can help. We can learn a skill for removing well rooted, but unhelpful beliefs. Rational coping helps us to generate reality driven ideas: the kind that promote our emotional well-being, increase our frustration tolerance, and make it less likely for us to get or stay upset.

The purpose of the rational coping statements for anxiety is to stop anxiety, and do it by reinforcing something rational. When these self-statements are practiced and learned, your brain takes over automatically. This is a form of "conditioning," meaning that your brain chemistry (its neurotransmission) actually changes as

a result of your new thinking habits. Pick only two or three statements, from the list that follows, that you like.

Pick the statements that make most sense and will help you stay rational, and repeat them to yourself out loud each day. You don't have to believe them fully yet – that will happen later.

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Statements to use when Anxiety is Near

1. I'm going to be all right. My feelings are not always rational. I'm just going to relax, calm down, and everything will be OK.
2. Anxiety is not dangerous – it's just uncomfortable. I am fine; I'll just continue with what I'm doing, or find something more active to do.
3. Right now, I have some anxious feelings I don't like. They are really just caused by adrenaline, however, and I can slow them down by calming myself. I will be fine.
4. I've stopped my negative thoughts before and I'm going to do it again now. I am becoming better and better at deflecting these Automatic Negative Thoughts and that makes me happy.
5. So I feel a little anxiety now, so what? It's not like it's the first time. I am going to take some nice deep breaths and keep on going. This will help me continue to get better.

Modeling Techniques

The therapeutic use of modeling is based chiefly on the work of **Bandura** (1969, 1971, 1976, 1977, 1986, 1997, 2007). Modeling as a therapeutic technique occurs when a client observes the behavior of another person and makes use of that observation. Learning how the model performs the behavior and what happens to the model as a consequence of learning the behavior are both a part of the technique.

In behavior therapy, the **five basic functions** of modeling (Spiegler & Guevremont, 2010, p. 267) are teaching, prompting, motivating, reducing anxiety, and discouraging.

Modeling can occur by **teaching** through demonstration—for example, watching someone throw a baseball or peel an apple. Modeling can serve as a **prompt**, such as when a child walk like a drum major, imitating his behavior. By reinforcing modeling behavior, people can **motivate** others to perform that behavior, such as when a parent makes a game of cleaning a room, so that the child can see how the task can be enjoyable. **Anxiety reduction** can occur as a result of modeling, such as when a child goes into the water after having watched another child do so, thus reducing a fear of the water. Last, an individual can be **discouraged** from continuing behavior, such as when a smoker watches a graphic film of a patient smoking and gradually dying from lung cancer.

In this section, **these five functions are combined** to varying degrees in live, symbolic, participant, and covert modeling.

Live modeling.

Basically, live modeling **refers to** watching a model, sometimes the therapist, perform a specific behavior. Often the modeling is repeated a number of times, and then, after having observed the modeling, the client repeats the observed behavior several times.

In Jones's (1924) study cited earlier, Peter's fear was reduced by observing other children modeling non-anxious behavior as they played with a rabbit.

Symbolic modeling. Often a live model is not available or would be inconvenient, so symbolic modeling is used. Common examples of symbolic modeling are films or videotapes of appropriate behavior; individuals are observed indirectly rather than in person. Other examples include photographs, picture books, and plays. For example, children's books about a child going to a hospital for an operation serve as symbolic modeling and can reduce a child's anxiety about surgery.

Self-modeling. Sometimes it is helpful to videotape/record a client performing the target behavior in a desired way (Dowrick, 1991; Dowrick, Tallman, & Connor, 2005). By filming a child interacting in a socially appropriate way with other children and then showing that film to the child, the child can observe himself modeling socially appropriate behavior and replace inappropriate behavior with the newly learned social skills.

Participant modeling. Sometimes it is helpful for the therapist to model a behavior for the client and then guide the client in using the behavior-participant modeling. If a client is afraid of climbing ladders, the therapist can model the behavior by first climbing the ladder. Then, using an adjoining ladder, the therapist can help the client climb a ladder while offering encouragement and physical support when necessary.

Covert modeling. Sometimes, when a model cannot be observed, it may be helpful to have a client visualize a model's behavior. In this process, covert modeling, the therapist describes a situation for the patient to imagine. Krop and Burgess

(1993) give an example of covert modeling with a 7-year-old deaf girl who was sexually abused by her stepfather. As a result of the abuse, the girl was inappropriately touching males (in the crotch area Bottom b/w legs sex organ area), engaging in other inappropriate sexual behavior, and having tantrums. In using covert modeling, Krop and Burgess had the girl imagine another little girl named Sara who felt good about making decisions not to throw tantrums and instead to interact appropriately with other children. Several scenes involved taking constructive action rather than acting out in a negative way.

Modeling, whether symbolic or live, is often used with other behavioral strategies to bring about change. In particular, modeling is frequently used in situations that involve interpersonal communication. Wolpe (1990) and many other behavior therapists have modeled appropriate assertive behavior with clients who are overly polite, have difficulty expressing negative feelings, or feel they do not have a right to express feelings. Because assertiveness skills are different, depending on the situation, behavior therapists often model and have their clients practice a variety of situations (Spiegler & Guevremont, 2010). Although assertiveness is perhaps the most common social skill to which behavior therapists have applied modeling, other social skills such as playing, negotiating, and dating are appropriate for modeling techniques. Such modeling behavior can also be used in cognitive-behavioral approaches that require individuals to observe events and then tell themselves how to perform appropriately.

Cost Benefit Analysis- CBA

The Cost Benefit Analysis (CBA) is a standard tool used in Cognitive Behavioral Therapy when people are weighing the pros and cons of change. It is also sometimes referred to as a decisional balance sheet. We include both a blank and a sample CBA.

It is important to write down both the pros and the cons of the behavior that you want to change. If you only write down the negatives then the positives will stay in your subconscious and work against you. This is why it is important to be conscious of both the positives and the negatives of the problem behavior.

A cost-benefit analysis is not specifically a CBT technique. It is used by CBT therapists to help clients examine and manage thinking and emotions in a natural process of problem solving. We all reflect on the pros and cons of our options when doing something new, e.g. going to a first date (or back for the second), accepting a job offer, renting or buying a flat, smoking, losing weight, going to a social event, moving somewhere, finding a CBT therapist. The cost-benefit analysis helps CBT therapists identify sticking points.

Advantages of Using a Cost benefit Analysis in CBT

- ☐ Excessive dwelling or worrying stops us from making decisions and can lead to feeling overburden, postponement or avoidance. Helps us to objectively define the problem, sometimes the hardest task of all;
- ☐ May require some backtracking if we have been worrying for a while.

A worry process can lead us down a long track of unsettling predictive cognitions (our “What if...” thoughts of hypothetical thinking).

□ Asking ourselves actually “what am I worried about?” brings us back to the source concern.

Problem-solving: asking, “what are all my options?” and considering their consequences can be illuminating.

□ When we are anxious or depressed, emotions can skew our thinking process – how we perceive events, store and recall memories.

A cost-benefit analysis can bring order to our thinking: writing things down on the page stops mixing and jumping thoughts from one worry to another.

□ This brings a helpful element of objectivity that can interrupt overgeneralizing adverse events and minimizing positive experiences, a common thinking style in depression Cost-benefit analysis:

□ In anxiety we tend to have an orientation towards identifying negative predictions and overestimating threats when considering consequences, i.e. when we are feeling anxious, we generate anxious thoughts, anxious thoughts make us feel anxious.

The cost-benefit analysis prompts us to look at other outcomes promoting an element of objectivity, by ‘getting it out of our heads’.

What underlying process is addressed in a cost-benefit analysis?

These difficulties are often rooted in our earlier experiences. From childhood and in adulthood, we all develop natural and healthy

strategies for coping with what life has to throw at us; these are called our “rules for living”.

Rules, in CBT are behaviours that are put into action when our assumptions are activated, but sometimes our assumptions can become inflexible and unhelpful.

Modifying assumptions: If I have an assumption that: “I always need to be prepared”, then this cognition may lead to developing an extreme rule for excessive planning. “I must find the perfect solution” then behaviourally I may tend towards the unobtainable perfectionistic traits. “I won’t cope if I am not prepared” then with this conditional thought I might develop intolerance of uncertainty, leading to an infinite search for certainty (an internal behaviour of worrying).

We can also get stuck with ineffective problem solving when we allow our emotions to be our guiding light to what we do, our behaviours. If I make my decision by emotional reasoning, acting on how I feel, sometimes very little will actually get done. For example, low mood, fear, guilt and shame can be normal reactions to a given situation, but if these are to be avoided (at all costs), then I may avoid thinking how to fix it, avoiding problem solving.

Example A pleasant man that presented with depression had many failed relationships around him. Becoming isolated made him sad because he valued and wished to be with people. When small issues happened (e.g. cancelling at the last minute, not paying a loan back, etc.) he never addressed them because it felt unconformable. The ‘minor’ events or considerations continued with some getting bigger (because he assumed that others did not know he was offended) but he felt resentful and would withdraw.

To cope with his problems he moved to London, believing he would make new friends. This avoidance and withdrawal led to family, friends, and romantic relationships eventually failing out. Taking the problem with him and starting work in London prevented him from developing new relationships in a busy new city. In this example, balancing the consequences of talking to people about how he felt in the short-term against the long-term would have helped with the emotional reasoning that took him to London. Avoidance might provide some short-term relief, but in the long-term, we may risk losing what we value most.

How it can help

- ☐ A cost-benefit analysis can help to unblock many types of unhelpful behaviours such as rituals, impulsive behaviours, and procrastination.
- ☐ Identifying the cognitive and behavioural factors that maintain difficulties sometimes lead naturally to what we need to do next.

Completing a Cost-benefit Analysis

- ☐ The cost-benefit analysis can be as simple as drawing a line down the middle of the page and writing pros on one side and cons on the other, then brainstorming everything that comes to mind after that.

Conduct a cost-benefit analysis:

- ☐ Do you think that taking it personally motivates you to try harder, or does it protect you in some way?
- ☐ Do you think that personalising the event/situation is "realistic"?
- ☐ What thoughts, feelings and behaviour would change if you personalized your experience less?

- Examine evidence for and against your personalizing statement.
- What is the quality of the evidence that supports your belief?
- We need to consider the advantages/benefits against the disadvantages, but this form helps us to break elements down further into a time frame (short and long-term), and also to examine how we ourselves and other people affect what is happening.
- Frequently cognitive errors or excessive rules lead to ineffective problem-solving by a focus on irrelevance.
- What is good about this form is that it restricts us to six points, for each domain. If I'm looking for the perfect solution, I will be locked in, analysing all the infinite options that are not the drivers for making a conclusive decision.
- There is no right or wrong decision because decisions are driven and based on what is important to us at any given time.
- Our decisions are contextual, in that they are formed on the basis of our own experiences, usually steered by our beliefs about how we see ourselves, others, the world and the future. However, sometimes it is useful to do some 'research' beforehand.

How to Use a Cost-benefit Analysis

- When we decide to do something, we consider many options, sometimes by discussing with friends, sometimes researching the internet, so on and so forth. Eventually, we think over the options, manipulate it all up, and we do what feels right. It is often useful to consider what information we need to assemble before we start to chew over our options.
- Any research needs to be limited by planning a start time and end time. Equally, as you sit down to complete your cost-benefit analysis, plan a dedicated stop signal – duration, emotion, time, event. If we cannot decide within the time, we should think about who we can consult for advice with this particular problem. This could be a family friend, work colleague or CBT therapist!

Thinking style: Be aware of your thinking styles, e.g. if I know I have perfectionistic tendencies then I need to be aware of what are realistic standards.

Likewise, for perfectionists, it will be important to plan what is “good enough” within a reasonable amount of time.

- We need to remember it is not about getting this right but getting it written; just do it. Resist returning to the analysis of the analysis.
- The CBT therapist reviewing a completed cost benefit analysis draws out the themes and directs our cognitive behavioural therapy path.
- Often, pros and cons are weighted in opposite corners. Usually, people find a direction that is ‘obvious’, but it is always helpful to investigate how the cost-benefit analysis helped us to unblock, and what needs doing to address this
- Equally, if the analysis is not leading to a solution, this can be revealing.
- Usually blocking thoughts and behaviours are connected with the endless searching for the perfect solution or needing to know what I don’t know.
- CBT is not a cluster of techniques, but a line of enquiry based on a rational theoretical conceptualization and principle that we know are effective.
- You cannot have behavioural change without cognitive change, and you cannot have cognitive change without behavioural change.
- In this sense, the cost-benefit analysis is not a treatment technique, but a means to an end. It can be one of the many stepping stones to overcome an anxiety disorder or a depressive episode.

● ***Doing cognitive homework***

REBT clients are expected to make lists of their problems, look for their absolutist beliefs, and dispute these beliefs. They often fill out the REBT Self-Help Form. They can bring this form to their therapy sessions and critically evaluate the contention of some of their beliefs.

Homework assignments are a way of tracking (chasing) down the absolutist “shoulds” and “musts” that are part of their internalized self-messages.

Part of this homework consists of applying the A-B-C model to many of the problems clients encounter in daily life. Work in the therapy session can be designed in such a way that out-of-office tasks are feasible and the client has the skills to complete these tasks.

In carrying out homework, clients are encouraged to put themselves in risk taking situations that will allow them to challenge their self-limiting beliefs.

For example, a client with a talent for **acting** who is afraid to act in front of an audience because of fear of failure may be asked to take a small part in a stage play. The client is instructed to replace negative self-statements such as “I will fail,” “I will look foolish,” or “No one will like me” with more positive messages such as “Even if I do behave foolishly at times, this does not make me a foolish *person*. I can act. I will do the best I can. It’s nice to be liked, but not everybody will like me, and that isn’t the end of the world.”

The theory behind this and similar assignments is that clients often create a negative, self-fulfilling prophecy (prediction) and actually fail because they told themselves in advance that they would.

Clients are encouraged to carry out specific assignments during the sessions and, especially, in everyday situations between

sessions. In this way clients gradually learn to deal with anxiety and challenge basic irrational thinking.

Making changes is hard work, and doing work outside the sessions is of real value in revising clients' thinking, feeling, and behaving.

Because therapy is seen as an educational process, clients are also encouraged to read REBT self-help books, such as *How to Be Happy and Remarkably Less Disturbable* (Ellis, 1999); *Feeling Better, Getting Better, and Staying Better* (Ellis, 2001a); and *Rational Emotive Behavior Therapy: It Works for Me—It Can Work for You* (Ellis, 2004a). They also listen to and evaluate tapes of their own therapy sessions. Making changes is hard work, and doing work outside the sessions is of real value in revising clients' thinking, feeling, and behaving.

PROSELYTIZING (others) pro.seli.tizing

One of the regular REBT cognitive techniques that you can use with your resisting clients is to encourage them to use REBT techniques with other people as well as themselves. For example, as you work on convincing clients that they partly create their own rage, you can give them homework assignments of trying to talk their friends or relatives out of their raging.

By helping show other people in their lives how they enrage themselves, your clients will more easily be able to talk themselves out of their own raging. This by no means always works, but some of my clients have been very successful in proselytizing and actually talking their friends and relatives out of some of their self-disturbances. When they refer them to me for psychotherapy, I can see that they have been considerably aided by my referring client.

Teaching other people how to use REBT on themselves can help your clients to deconstruct their own disturbances. As John

Dewey said many years ago, we learn by teaching others to learn. Some of my clients do so well with using REBT with their friends and relatives that they actually help these people become considerably less afflicting.

Reframing

Reframing is another strategy for getting bad events into perspective is to re-evaluate them as 'disappointing', 'concerning', or 'uncomfortable' rather than as 'awful' or 'unbearable'. A variation of reframing is to help the client see that even negative events almost always have a positive side to them, listing all the positives the client can think of (NB: this needs care so that it does not come across as suggesting that a bad experience is really a 'good' one).

"Cognitive reframing" describes a process in which we change the way we look at things. When we view things negatively or in a negative frame, there can be harmful effects such as increased stress, and symptoms of anxiety or depression.

The objective for people in therapy is to reframe certain situations so that they are no longer affected by that negative state of mind.

When life presents us with a situation, we usually decide what meaning the situation has for us almost immediately. That meaning is the "frame" we give it. However, we don't have to continue viewing it in that frame. Instead, we can give it a different meaning by changing the way we think and feel about it. Perspective is, largely, a choice.

Cognitive reframing is a technique used to shift your mindset so you're able to look at a situation, person, or relationship from a slightly different perspective. Cognitive reframing is something that you can do at home or anytime you experience distorted thinking. It can sometimes be helpful to have a therapist's assistance, particularly if you are caught in a negative thought pattern.

The essential idea behind reframing is that the frame through which a person views a situation determines their point-of-view. When that frame is shifted, the meaning changes and thinking and behavior often change along with it.

Another way to understand the concept of reframing is to imagine looking through the frame of a camera lens. The picture seen through the lens can be changed to a view that is closer or further away. By slightly changing what is seen in the camera, the picture is both viewed and experienced differently.

Techniques of Cognitive Reframing

Reframing may be used to change the way people think, feel, and behave. Here are a few examples of how reframing may be used in therapy.

Family Therapy

In a family therapy session, **Carla complains bitterly that her mother** is overly involved in her life, constantly nagging her about what she should be doing. In attempting to shift Carla's negative view of her mother, the therapist offers this reframe: "Isn't it loving of your mother to teach you ways to take care of yourself so you'll be prepared to live on your own without her?"

Individual Therapy

A person in individual therapy is struggling to accept the limitations of having a chronic illness. The therapist attempts to reframe how they view their illness by saying, "Can you think of your illness as a built-in reminder to take care of your health throughout your life?"

A man is upset that he wasn't chosen for a promotion. The therapist asks him what positive things could come from not being promoted. The man might note that the new job came with some unwanted additional stresses and that he might be able to work toward another role that is better suited to his needs and long-term career goals.

A woman is upset about getting a ticket for texting while driving, so her therapist talks about the dangers of texting while driving. Eventually, she is able to see that the ticket might help prevent her from engaging in the dangerous behavior again in the future.

Cognitive reframing can be useful for people who are experiencing mental health conditions, but it can also be helpful for improving overall mental well-being. Using cognitive reframing can help you become more positive and resilient in the face of life's challenges.

What are some examples where cognitive reframing works?

In situations where a person is feeling sad, they can change how they view the situation so that they can focus on the things that are going right. When people are experiencing stress, rather than getting overwhelmed by the things they cannot change, reframing can help them focus on the aspects of the situation that they can control.

Thought Stopping

Thought stopping is a strategy that involves blocking and replacing unwanted, distressing thoughts. The technique used in cognitive-behavioral therapy (CBT) as a way to halt or disrupt negative thoughts. Then, a more adaptive or helpful thought can then be substituted for the unhelpful one.

Thought stopping has a long history of use in CBT. However, some research suggests that when not done correctly, it is ineffective and can often contribute to worsening anxiety or obsessive thinking.

This article explores how thought stopping works, why it is sometimes ineffective, and when it can be useful. It also covers some of the different thought stopping techniques that people might use, and what strategies might be better to try instead.

Also known as thought suppression, the idea is that by repeatedly preventing the brain from thinking about certain things, the thoughts will fade out of awareness and no longer come to mind. While our ability to think—to problem-solve, pay attention, remember, cherish, and create—is a tremendous asset that allows us to survive and flourish, our ability to think can also work against us. Overthinking can exacerbate stress and contribute to mental health disorders like anxiety and depression. It can prevent us from living fully and enjoying inner peace and satisfaction. It's natural to want to stop thinking certain thoughts or overthinking in general.

How Thought Stopping Works

The focus on thought stopping is to disrupt, dismiss, and replace the unwanted thought. Some of the different strategies that you might use to do this include:

- Saying "Stop!" inside your mind
- Holding a visualization in your mind whenever you have the thought
- Clapping your hands or snapping your finger whenever the thought enters your mind
- Making a checkmark on a piece of paper every time you have the thought

The idea behind this approach is that these actions help you notice how often you are having the thought and help distract you from it. Another purpose is to help you notice some of the triggers that might be present when you do have those unwanted or intrusive thoughts.

In the past, people were sometimes told to put a rubber band around their wrist and snap it every time they had an unwanted thought.

It is also important to note that simply trying to suppress the thought is not the goal and can cause unwanted thoughts to rebound and worsen. Research has also found that if you stop the thought without replacing it with a more positive one, negative thoughts tend to increase.

Thought stopping is more about noticing these thoughts and then gently redirecting your mind to a more helpful, positive one.

When Is Thought Stopping Used

Thought stopping has often utilized as a way to **treat many** different problems, including:

- Agoraphobia
- Anxiety
- Catastrophic thinking
- Intrusive thoughts
- Obsessive-compulsive behavior
- Rumination disorder
- Social anxiety

One way that thought stopping might be helpful is if you are in a situation where you cannot address an unwanted thought. If you need to avoid thinking about something during a critical moment, temporarily using thought stopping can allow you to redirect your mind until you are in a time and place where you can deal with it effectively.

Some research also suggests that thought stopping can be one component of a treatment plan that can help improve the quality of life for patients with adjustment disorders after cancer treatment. Adjustment disorders before, during, and after cancer treatment can be complex mental issues, which require professional guidance for successful treatment.

Is Thought Stopping Effective?

While some therapists and group therapy programs promote thought stopping as an effective technique, the results of a 2010 review of research by a group of Yale psychologists disagree. The review found an association between thought suppression strategies and greater depression and anxiety, not less. The psychologists found an association between anxiety reduction and other strategies, including cognitive restructuring, acceptance, and problem-solving techniques.

Some experts suggest that thought stopping can be harmful when used to treat obsessive-compulsive disorder (OCD). It can increase obsessive thoughts and become a difficult to overcome ritual.

When Might Thought Stopping Be the Right Approach?

In 2012, authors of a review in *Clinical Psychology Review* analyzed different types of studies into thought stopping. They looked at whether those studies involved people with and without diagnosed mental health disorders. It appears that in the absence

of a mental health disorder such as OCD, and in cases where the thoughts aren't severely disruptive to life, thought stopping might be safe to try.

Additionally across studies, participants experienced different degrees of success with thought stopping. In other words, not everyone experienced an intense or bothersome rebound effect, thus leading researchers to conclude that thought stopping should be an individualized approach.

Thought Stopping Techniques

While not all therapists recommend the use of thought stopping, learning to become more aware of your negative thoughts can be helpful. When used in this way, thought stopping can be a tool that helps you identify problematic thoughts and learn ways to replace those thoughts. Some thought stopping techniques that can be useful include:

1. **Stop the thought:** Tell yourself "stop" or try visualizing a stop sign in your mind. You might also envision yourself capturing the thought inside of a net where you can contain it in order to work through it later.
2. **Notice the thought:** Don't try to suppress the thought. Acknowledge it exists without dwelling on it.
3. **Replace the thought:** Now that you have evidence that the thought is not a reflection of reality replace it with something more helpful. This might be a self-affirmation or mantra that helps keep you feeling optimistic and focused.
4. Doing something simple that instantly creates an irritating sensation (e.g., snapping a rubber band on your wrist)

When you have more time to focus on the thought, work on actively interrogating and disputing it. What other alternatives are there? Is the thought realistic? What might disprove it? Are

cognitive distortions such as all-or-nothing thinking or black-and-white thinking affecting how you view a situation?

Other techniques that might be helpful when you are trying to stop a thought include:

- **Set a timer:** Give yourself a set amount of time (just a minute or two) to have the thought. Once the alarm goes off, tell yourself, "Stop!" and redirect your mind to something else.
- **Use relaxation techniques:** If you find yourself having a negative thought, use a relaxation technique such as progressive muscle relaxation or deep breathing to refocus your attention elsewhere.
- **Meditate:** Halt the negative thought by trying a quick meditation to shift your focus to more positive thoughts.

Stopping negative thought cycles

There are helpful and unhelpful ways of reacting to a situation, often determined by how you think about them.

For example, if your marriage has ended in divorce, you might think you've failed and that you're not capable of having another meaningful relationship.

This could lead to you feeling hopeless, lonely, depressed and tired, so you stop going out and meeting new people. You become trapped in a negative cycle, sitting at home alone and feeling bad about yourself.

But rather than accepting this way of thinking you could accept that many marriages end, learn from your mistakes and move on, and feel optimistic about the future.

This optimism could result in you becoming more socially active and you may start evening classes and develop a new circle of friends.

This is a simplified example, but it illustrates how certain thoughts, feelings, physical sensations and actions can trap you in a negative cycle and even create new situations that make you feel worse about yourself.

CBT aims to stop negative cycles such as these by breaking down things that make you feel bad, anxious or scared. By making your problems more manageable, CBT can help you change your negative thought patterns and improve the way you feel.

CBT can help you get to a point where you can achieve this on your own and tackle problems without the help of a therapist.

Self-Monitoring

Self-monitoring is a technique in which cognitive behavioral therapists almost always teach their clients. It is a form of data-gathering in which clients are asked to systematically observe and record specific targets such as their own thoughts, emotions, body feelings, and behaviors. For example, a depressed client might be asked to record information about situations where their mood felt particularly low, a client recovering from an addiction problem might record what they were doing and thinking just prior to using substances, or a client suffering from post-traumatic stress disorder might complete a diary to record details of their trauma flashbacks.

One aim of CBT is to help people to understand how what they think and do affects the way they feel. The process of self-monitoring can help clients better appreciate the links between situations, thoughts, emotions, body sensations, and their responses.

Self-monitoring can provide a simple means to track client progress (e.g. measuring the frequency and intensity of intrusive thoughts over the course of therapy) as well as readiness for different stages of a treatment intervention (e.g. the introduction of cognitive restructuring or behavioral experiments).

Although self-monitoring is conceptually fairly simple, it is often challenging for clients to conduct effectively (Korotitsch & Nelson-Gray, 1999). To support clients with self-monitoring, we have outlined below a number of best practices that can be followed.

What is self-monitoring in therapy?

Self-monitoring is a practice in which clients are asked to systematically observe and record specific targets such as thoughts, body feelings, emotions, and behaviors. It is part of a wider practice of empiricism and measurement that is integral to CBT (Persons, 2008), and it functions as both an assessment

method and an intervention (Korotitsch & Nelson-Gray, 1999; Proudfoot & Nicholas, 2010).

Self-monitoring is comprised of two parts – discrimination and recording (Korotitsch & Nelson-Gray, 1999).

During discrimination, the client is trained to bring their awareness to target phenomena by identifying and noticing when they occur. For many clients, bringing attention and awareness to specific thoughts, feelings or emotions will be novel and difficult, but improving clients' awareness of their symptoms and behavior is a critical component of CBT.

Recording consists of documenting occurrences (usually through a written record). Creating a record makes discrimination an explicit and conscious process that can be reviewed and analyzed. It also means that – through documentation of specific events – additional detail or context can be included and analyzed, improving the client's understanding of their symptoms, as well as the sequence and process by which they occur. Discrimination and recording are skills that need to be learned and improved. Clients are likely to need support and training to complete self-monitoring records accurately.

Self-monitoring is everywhere across CBT, with “self-monitoring procedures... described and recommended within most empirically supported treatments” (Korotitsch & Nelson-Gray, 1999). Therapists and clients may not always label it as self-monitoring, and may instead refer to the use of diaries, logs, records, schedules, or symptom monitoring. Commonly used examples are activity diaries for behavioral activation, logs of eating behaviors for the treatment of eating disorders, and thought records for capturing negative automatic thoughts.

Why practice self-monitoring in CBT?

“Self-monitoring puts clients in charge of empirically examining their beliefs and behavior.” (Cohen et al, 2013)

CBT is an open (non-mysterious) therapy in which the client is an active participant, and where the goal is to help clients to develop skills to manage or overcome their difficulties.

Self-monitoring is a straightforward way to introduce clients to the concept of active participation in therapy, and it supports clients' engagement and motivation by fostering a sense of self-control and autonomy (Bornstein, Hamilton & Bornstein, 1986; Proudfoot & Nicholas, 2010).

By completing self-monitoring records away from therapy sessions, clients gain additional feedback that serves to reinforce and consolidate the work completed in sessions (Bornstein, Hamilton & Bornstein, 1986).

Self-monitoring helps clients to develop a critical awareness of their difficulties, which prepares them for change: "awareness is a logical first step of the change process" (Persons, 2008). For example, self-monitoring the frequency of angry outbursts can help clients to see the true scale of the problem (that they shout at their partner nearly every day, not once every fortnight as they initially reported). Self-monitoring the time taken to complete compulsive rituals can help a client with OCD to see how much time they lose each day, and can help to build motivation to engage in therapy.

As skills in discrimination improve, self-monitoring can be refocused to help clients better understand the links between situations, thoughts, emotions, body sensations, and their responses.

An additional benefit of self-monitoring is that, in some circumstances, it has been shown to have a modest but beneficial treatment effect – so called 'reactive' effects (Korotitsch & Nelson-Gray, 1999; Proudfoot & Nicholas, 2010). The act of self-monitoring has been shown to increase the frequency of positive behaviors and decrease the frequency of negative behaviors (e.g.

for eating disorders or substance misuse; Korotitsch & Nelson-Gray, 1999).

When should self-monitoring be practiced in therapy?

Self-monitoring is often taught early in therapy, during the assessment stage. It can be particularly useful when the target phenomenon is covert and cannot be observed by anyone but the client themselves (Cohen et al, 2013), for example, negative automatic thoughts, body sensations or self-harm.

Since self-monitoring is both a form of assessment and an intervention (Korotitsch & Nelson-Gray, 1999; Proudfoot & Nicholas, 2010), the format and nature of self-monitoring is likely to change during therapy. Self-monitoring may initially focus more on capturing data to help the client and therapist identify and prioritize problems. Subsequent data might be used to inform a client's formulation – for example, monitoring triggers and environmental factors to identify causal relationships to the problem.

Once an intervention plan has been developed, self-monitoring can be used to track the plan's implementation away from therapy sessions, such as monitoring the frequency and success with which a new adaptive coping strategy is used (Cohen et al, 2013). Finally, self-monitoring might be used throughout therapy as a method of measuring the effectiveness of treatment as a whole and whether therapy goals have been met.

How is self-monitoring conducted?

Events can be 'monitored' retrospectively, with the client and therapist working together in-session to review the client's memory for an event, and exploring their thoughts, emotions, and behaviors. The problem to this approach is that retrospective records are affected by recall biases, so salient elements are particularly likely to be recalled (such as body feelings), whereas

other details are likely to be lost or recalled inaccurately (such as contextual triggers or preceding thoughts; Proudfoot & Nicholas, 2010). The advantage self-monitoring is that the client can record information close to (or at the time of) an event and this often results in a richer, more complete and accurate account (Bornstein, Hamilton & Bornstein, 1986).

What are common targets of self-monitoring in CBT?

Targets for self-monitoring are personal, depending upon the client's presentation. Target domains include:

- **Events:** e.g. monitoring events in which a client feels a certain way.
- **Emotions:** e.g. monitoring situations in which particular emotions are experienced.
- **Thoughts:** e.g. monitoring the occurrence, frequency, and content of negative thoughts.
- **Memories:** e.g. monitoring the occurrence, frequency, and content of unwanted memories.
- **Body sensations:** e.g. monitoring the occurrence, frequency, triggers, context and consequences of physiological feelings that concern the client.
- **Attention:** e.g. monitoring situations where a client becomes particularly self-focused, or especially threat-focused.
- **Activity:** e.g. monitoring daily levels of activity in depression, or monitoring the relationship between activity, enjoyment, and mastery.
- **Behavior:** e.g. monitoring behaviors such as avoidance, self-harm, safety behaviors, bingeing and purging.

How to train clients in self-monitoring in CBT

Self-monitoring is a skill that develops with time and training, so clients may need support to complete self-monitoring records accurately. Elements of effective training include:

- **Select a relevant target for monitoring** – will data about this influence clinical decision making?
- **Ensure the target is specific, clearly defined and agreed upon** – does the client understand what they are monitoring and why?
- **Provide clear instructions** – does the client understand how to complete self-monitoring? Can they refer back to instructions without the therapist?
- **Offer concrete support for recording** – how will recording be completed? Does the client have the necessary resources and materials?
- **Provide modeling and training in how to record** – has the client practiced recording? Has the therapist observed the client practicing?
- **Use the monitoring data** – subsequent sessions should always review and draw upon prior self-monitoring data the client has recorded.

It can be difficult to ensure that records are completed in a timely manner, but a number of techniques can be used to encourage regular, accurate self-monitoring. The manner and format of self-monitoring can be adapted to suit the client's preferences. If clients have literacy difficulties – prepared forms that can be easily marked or ticked are preferable. If they feel conspicuous using a printed sheet, they may be more comfortable filling out a form on a smartphone, or keeping the sheet inserted into a small notebook. Psychoeducation can be used to stress the importance and benefits of self-monitoring (e.g. the development of skills to manage distressing emotions). For children and young people, extrinsic rewards can be used when self-monitoring records are completed (Cohen et al, 2013). More practically, smartphones, computers and watches can be used to set reminders (Proudfoot & Nicholas, 2010).